

ENGLISH FOR WORK / SEPTEMBER 2026

Healthcare Administration English

Vocabulary & phrasebook

Keep precise meanings and useful expressions close. Retrieve the words, then use them in a complete message.

PRACTICE THAT TRANSFERS TO WORK

Read a realistic case. Find the right language. Speak, write, get feedback, and try again.

Eight lessons | Intermediate to advanced | Independent or classroom study

For hospital administrators, clinic managers, practice administrators, care coordinators, patient-experience leaders, revenue-cycle staff, operations managers, and healthcare-adjacent professionals.

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

All scenarios, figures, and model responses are fictional teaching material. Use invented details in practice.

[Open this course on English Ladder](#)

Make vocabulary usable

Knowing a definition is a start. To use a term well, explain it in plain English, connect it to a case, and choose a natural sentence around it.

- Read five terms, cover their definitions, and recall the meanings.
- Sort a pair you might confuse and explain the difference.
- Use one term in a sentence about a fictional case.
- Ask a partner to explain the sentence without repeating the specialist word.
- Return to the same terms on another day and test recall again.

Abbreviations need context

Expand an abbreviation on first use when the listener needs it. Some abbreviations have different meanings across professions: API can refer to an application programming interface or an active pharmaceutical ingredient. The course context determines the meaning.

Definitions have limits

These are concise language-learning explanations, not complete legal, clinical, or technical specifications. Use the applicable authoritative definition for regulated or operational decisions.

Field vocabulary A-Z

accreditation

External recognition that an organization meets defined quality, safety, or professional standards.

audit trail

A record showing who changed what, when, and often why.

authorization

Required approval from a payer, regulator, manager, or system before a service, transaction, or action can proceed.

bed management

Coordinating hospital bed availability and assignment in relation to patient needs.

business associate

Under HIPAA, certain persons or organizations performing covered functions or services involving protected health information.

care gap

A difference between recommended or intended care and the care a person has received.

census

The number of patients counted in a facility or unit at a defined time.

claim

A statement that may need evidence, approval, qualification, or disclosure before it is used externally.

clean claim

Insurance claim submitted with the required data and documentation so it can be processed without avoidable delay or rejection.

coding

Assignment of standardized diagnostic, procedure, or service codes used for records, billing, reporting, or reimbursement.

corrective action

Action taken to fix a current problem and prevent recurrence.

dashboard

Visual summary of selected measures used to monitor status, performance, or risk.

denial

Decision by an insurer, payer, regulator, or authority not to approve, pay, or grant a requested item or claim.

discharge planning

Coordinating the needs and arrangements for a patient leaving a care setting.

eligibility

Rules that determine whether a person, account, provider, service, or request qualifies for a program, benefit, or action.

grievance

A formally raised concern or complaint handled through an established process.

HCAHPS

US standardized patient-experience survey used by hospitals for public reporting and quality improvement.

HIPAA

US health privacy framework governing protected health information in covered health contexts.

KPI

Key performance indicator used to monitor progress against an important objective.

minimum necessary

HIPAA principle requiring use or disclosure of only the minimum protected health information needed for the purpose.

near miss

An event that could have caused harm but did not result in the specified harmful outcome.

no-show

Scheduled patient, customer, or guest who does not arrive or cancel in time for the appointment or reservation to be reused.

owner

Named person or role accountable for a decision, action, deliverable, or risk.

patient experience

A person's experience of interactions and services across their care.

protected health information

Individually identifiable health information protected by HIPAA when held or transmitted by covered entities or business associates.

readmission

Return of a patient to a hospital or facility within a defined period after discharge, often tracked as an outcome measure.

referral

Order, recommendation, or routing process that directs a patient or case to another clinician, service, or specialist.

service recovery

Actions taken to address a service failure and restore the customer's experience.

social determinants

Social and environmental conditions that influence people's health and access to opportunities.

staffing ratio

The relationship between staff numbers and a defined workload or population.

throughput

Amount of work, patients, goods, cases, or transactions completed in a period of time.

variance

Difference between actual and expected performance, cost, timing, quality, or volume.

Expressions for this course

Complete the frames with the facts of your case. A frame helps organize meaning; it should not replace an actual explanation.

Ask a precise question

When you say ..., do you mean ...?

Could you clarify which ...?

So, my understanding is Is that right?

Name the exact word, fact, or requirement you need clarified. Ask one focused question at a time. Repeat your understanding and give the other person a chance to correct it.

Make a complex point clear

In this context, ... means

The difference is that

How would you explain that distinction to a colleague?

Start with the reader's question. Explain one unfamiliar term with familiar words, then give a relevant example or contrast. Check understanding without asking only 'Do you understand?'.

Give a useful status update

We have completed ...; ... remains open.

As of ..., the status is

The next update will cover

Lead with the current state, then the important change or open item. Use completed-action language only for completed work. Add a next update point when it is known; do not invent a date to make the message sound complete.

Match certainty to evidence

The available evidence shows

This may indicate ..., but

We have not yet established whether

Say what the evidence shows, identify what is still unknown, and limit the conclusion to that evidence. Words such as 'may', 'suggests', and 'in this sample' are useful when the uncertainty is real. Do not soften an urgent, verified safety concern.

Ask for an actionable next step

Could you provide ... so that ...?

Please confirm ... by [agreed time].

If that timing is not possible, please let me know

State the action and the information needed. Explain why it matters and specify a deadline only when one is given or agreed. 'Could you' is polite; repeated apologies can obscure a legitimate request. Urgent situations may require a direct request.

Repair a misunderstanding

My earlier message did not make ... clear.

I'm sorry for The correct information is

Let me check that the revised explanation addresses

Name what was unclear or wrong, acknowledge its effect, and correct it. A brief apology can help, followed by a practical next step. Avoid explaining away the other person's reaction or promising an outcome you cannot control.

Transfer information and responsibility

The current status is ...; the outstanding item is

The record shows

Please confirm who has accepted

State the current situation, relevant background, open task, and receiving role. Ask for acknowledgment or repeat-back of the critical item. Sending a message and transferring responsibility are not always the same thing.

Model responses in context

Read the situation first. Cover the model, say your own response, and compare the two for meaning and tone.

01 | Patient Access, Scheduling, and Referrals

A referral is received without the requested specialty or reason for the appointment. The patient is expecting a booking confirmation.

Could you confirm the specialty and referral details through the approved channel? I can explain the scheduling process to the patient, but I cannot confirm an appointment until the required information is available.

Try it again: The other person uses an ambiguous word such as 'ready', 'approved', or 'urgent'. Clarify it before continuing.

02 | Revenue Cycle, Coding, and Denials

A dashboard groups rejected submissions and denied claims together. A manager wants to understand where follow-up work is needed.

These categories describe different points in the process. Let's separate submissions that did not enter processing from claims that received a denial, then identify the follow-up required for each group.

Try it again: Your listener is new to this field. Replace two specialist expressions with clear explanations without changing the meaning.

03 | Patient Flow, Capacity, and Staffing

A unit reports 24 occupied beds and two planned discharges. The discharge timing is unconfirmed, but another team assumes two beds are available now.

There are 24 occupied beds. Two discharges are planned, but their timing is not confirmed. I'll distinguish current availability from expected availability in the capacity update.

Try it again: The listener asks you to reduce the update to two sentences. Preserve the most important completed and pending items.

04 | Quality, Safety, and Accreditation

A near-miss report identifies a scheduling error caught before the appointment. The review has not established how the error occurred.

The error was identified before the appointment. The cause is still under review. I'll record the sequence of events and the checks that caught it without assigning blame or an unconfirmed cause.

Try it again: Someone asks for a yes-or-no answer when the evidence supports only a qualified answer. Be concise while preserving the uncertainty.

05 | HIPAA, Privacy, and Information Governance

A colleague requests a full patient list for a limited administrative task. The appropriate scope and access have not been confirmed.

Could you specify the purpose and fields required? Let's confirm the permitted access and necessary information through our privacy process before sharing the list.

Try it again: The other person cannot meet the requested timing. Clarify an alternative without inventing authority to change a formal deadline.

06 | Patient Experience and Service Recovery

A patient has called twice about a delayed response. The administrator can investigate the communication delay but cannot make a clinical decision.

I'm sorry you have had to call again. I can check the status of your request and confirm the next contact. For the clinical question, I'll connect you with the appropriate care team.

Try it again: The listener remains disappointed after the correction. Acknowledge the impact and restate the available next step calmly.

07 | Population Health and Care Coordination

A discharge coordination note lists transport as pending. The next coordinator needs to know who is following up and what has been confirmed.

Transport remains unconfirmed. The request is recorded, and the assigned coordinator is following up. Please check the latest status with that person and record any change before telling the patient a departure time.

Try it again: The intended recipient cannot take ownership. Keep the status clear and identify the need for an authorized reassignment.

08 | Executive Dashboards and Board Updates

A board chart shows waiting time falling from 40 to 30 minutes, but the reporting period changed from monthly to weekly.

The reported waiting time is lower, but the periods differ. I'll label the time windows and prepare a comparable view before we describe the change as a sustained improvement.

Try it again: Your listener is new to this field. Replace two specialist expressions with clear explanations without changing the meaning.

Use the language responsibly

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

Meaning before performance

Use specialist terms only when they help the listener. Explain unfamiliar words, preserve uncertainty, and ask when an instruction or responsibility is unclear. The model responses illustrate language; they do not authorize professional actions.

Sources and further reading

The cases, workshops, and explanations are original teaching material. These references provide language frameworks and selected professional context. References were checked in September 2026; consult current local guidance for actual work.

Digital.gov: plain-language guidance

<https://digital.gov/guides/plain-language>

Council of Europe: CEFR descriptors

<https://www.coe.int/en/web/common-european-framework-reference-languages/cefr-descriptors>

HHS: HIPAA business associates

<https://www.hhs.gov/hipaa/for-professionals/privacy/guidance/business-associates/index.html>

AHRQ: SBAR communication tool

<https://www.ahrq.gov/teamstepps-program/curriculum/communication/tools/sbar.html>

Your next step

Choose one expression you can use in a genuine work situation. Rehearse it with fictional details, then adapt the wording to your role and audience.